

February 23, 2026

Thomas Keane, MD, MBA
Assistant Secretary for Technology Policy, National Coordinator for Health Information
Technology
Department of Health & Human Services
200 Independence Ave, SW
Washington, DC 20201

RE: RIN 0955-AA13

Dear Dr. Keane,

Introduction

Microsoft appreciates the opportunity to respond to the *Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care* from the Office of the Deputy Secretary and Assistant Secretary for Technology Policy (ASTP) and the Office of the National Coordinator for Health Information Technology (ONC) at the US Department of Health and Human Services (HHS). Microsoft's perspective reflects its broad experience across consumer health engagement, clinical productivity tools, enterprise AI infrastructure and federally compliant cloud and data services. This experience, grounded in Microsoft's Responsible AI principles of safety, transparency, fairness, security, and accountability, aligns closely with the Department's goal of deploying high value AI in a safe and transparent manner.

Microsoft is committed to supporting HHS as it shapes a regulatory ecosystem that allows AI solutions to enhance clinical care and promote public health. To that end, Microsoft recommends the following:

- **Regulation:** In furtherance of the OneHHS goal to standardize how federal agencies evaluate, adopt and interface with private AI innovations, the Department should consider publishing a guidance document for Industry that clearly establishes boundaries for AI use across clinical and consumer health domains. Clear definitions will enable the ecosystem of AI developers and AI implementers to fully govern AI. This governance context should span the structuring of AI-ready data and AI tools for development and deployment of AI in the health ecosystem. These

considerations should account for AI systems leveraging imaging, structured text, unstructured text, pathology and numerous other data types needed to derive insights that enable care teams and patients. Ultimately, the foregoing approach could assist in allowing developers to design safe consumer experiences, allow health systems to more confidently adopt AI solutions, and ensure that clinicians maintain control over medical decisions.

- **Reimbursement:** CMS should recognize AI-enabled products and services that measurably enhance clinical workflows and enable clinicians to improve patient outcomes as eligible for reimbursement.
- **Research & Development:** AI researchers working in the life sciences who receive funding from federal agencies (e.g., NIH, NSF, etc.) should be required to share datasets, synthetic data programs and evaluation infrastructure to support safe model testing and fair comparisons across AI tools.

Microsoft's Perspective

AI is already enhancing patient experiences by optimizing clinical workflows, assisting patient comprehension and generally improving the interactions people have with the healthcare system. For instance, Microsoft products such as Dragon Copilot and prior versions of Dragon technology have demonstrated measurable improvements in clinical workflows by streamlining documentation processes, reducing the time clinicians spend on administrative tasks and enhancing overall productivity. Quantitative analyses indicate that the adoption of Dragon Copilot has led to a reduction in clinical documentation by an average of 30-40%, enabling healthcare professionals to allocate more time to direct patient care. Additionally, user reports show a significant decrease in transcription errors and an improvement in the accuracy of patient records, with some institutions noting a 25% reduction in documentation-related discrepancies. These enhancements contribute to faster patient throughput and improved satisfaction for both clinicians and patients. The adoption of ambient scribe technologies like Dragon is only continuing to grow. Nonetheless, private sector stakeholders are facing the regulatory burden of navigating an uncertain and quickly changing regulatory landscape, hindering pathways to clinical AI adoption.

As generative AI tools become embedded in clinical environments, HHS should provide a unified federal interpretation that establishes clear regulatory expectations to maintain safety, trust and innovation in the context of health AI solutions. To successfully deploy safe and effective clinical AI, Microsoft believes federal level guidance is needed to enable the appropriate and safe use of AI-ready data. A unified federal interpretation of AI system

use should also address access to longitudinal patient records to ensure AI results are meaningful, establish a framework for the creation of a national Master Patient Index, and data standards development for areas beyond medical text such as radiology and digital pathology imaging. Additional means to achieve trust, safety and innovation could be achieved through driving alignment on AI transparency standards for model developers, such as a standard set of reporting indicators for performance.

Innovators and health systems need predictable and appropriately scaled guidance that differentiates low- and high-risk health AI tools; for example, differentiating between consumer wellness health AI tools from FDA-regulated medical devices or distinguishing between clinical decision support tools, which augment or assist but do not replace clinician judgment, from fully autonomous clinical AI systems. This guidance should align with recent FDA announcements that prioritize this philosophy, particularly the updated clinical decision support guidance and the general wellness policy for low-risk devices, that offer flexibility but recognize the increasing role of lifecycle-based oversight for adaptive systems.

As systems evolve through post-deployment learning, oversight must incorporate real-world monitoring, change control, transparent communication with users and guardrails that preserve clinical authority. The OneHHS guidance document should apply a risk-based framework to help classify healthcare AI solutions based on their potential impact and tailor oversight accordingly. Any such guidance should also reflect the real-world evolution of AI-enabled documentation assistants and intelligent workflow aides, such as Microsoft Dragon Copilot, which reduce clinicians' burden while ensuring that clinical decision-making remains squarely under their control and is expanding its reach to multiple personas, including Nurses and Radiologists. As noted above, ambient tools such as these directly address one of clinicians' biggest barriers to AI adoption, administrative overload and free up time for patient care while maintaining clinician oversight of notes.

Recommendations

Reimbursement

Uncertainty in payment policy is limiting the adoption of AI in clinical care – as providers may not know how to choose which solutions to invest in and whether they will eventually be reimbursed. CMS should create reimbursement pathways and define AI-enabled clinical services as a recognized class, to reflect the demonstrable value of AI.

Clinician-supervised interventions, such as AI-enhanced documentation, image review, care coordination and real-time clinical support should have explicit billing mechanisms

through Medicare. AI Device manufacturers should be compensated for measurable reductions in administrative burden, improvements in documentation quality and enhancements in care coordination and provider well-being.

Consumer-facing navigation and conversational AI agents should be eligible for coverage when they help clinicians to improve patient experience and outcomes. Imaging agents and diagnostic reporting AI can also reduce friction in report generation while increasing report accuracy, suggesting eligibility would benefit the ecosystem. CMS Innovation Center models can test or pilot these agents where necessary in accountable care and Medicaid settings.

Interoperability

Despite progress, the incomplete adoption of consistent standards for data extraction and use is a challenge to advance health AI in clinical care. Incomplete or inconsistent data limits the ability of AI applications to meaningfully interoperate with electronic health records, imaging platforms and other clinical systems. While Microsoft supports ASTP/ONC's efforts to advance to a leaner, interoperability-first certification approach emphasizing FHIR-based APIs, Microsoft encourages ASTP/ONC to also focus on standards that prioritize visible data-quality and provenance indicators. Consumer companions and clinical aides both depend on reliable data, labs, medications, imaging, longitudinal care plans and patient-generated information to support safe and accurate guidance, making a policy shift to require API access to all Electronic Health Information (EHI) and not just the United States Core Data for Interoperability (USCDI) elements, is imperative to empower patients with complete, usable access to their health information. We recommend mandating comprehensive export capabilities with appropriate documentation to enable patients and their authorized applications to access their complete health story.

Evaluation Methods

HHS should invest in shared benchmark datasets, synthetic data programs and evaluation infrastructure to support safe and transparent model testing and fair comparisons across tools, with an emphasis on post-market monitoring. Evaluation guidance should address usability, workflow alignment, subgroup performance, hallucination risk, failure mode testing and monitoring for model drift. Transparent evaluation guidance will create a consistent foundation for developers, health systems, and regulators while improving public trust.

Privacy

Privacy and liability should establish expectations for transparency, permitted data use, monitoring, security, and human oversight. Today, a patchwork of new state laws looking to

govern such data use create challenges to predictable approaches; clear federal templates would help drive clarity and reduce contracting friction while protecting patients and caregivers in the context of a safe, predictable environment for innovation.

Consumer AI Shift

Healthcare is rapidly shifting from the clinic to the home, where everyday consumer tools increasingly shape how people interpret symptoms, decide when to seek help, prepare for clinical visits, follow care plans and navigate an overwhelmingly complex system.

Consumer AI in Copilot and Bing is already the first touchpoint for millions of Americans, helping them make sense of their health long before they reach a clinician.

As this transition accelerates, federal policy should prioritize keeping low-risk conversational AI systems under a general wellness framework, as they provide education, navigation, and comprehension support -- not clinical triage or diagnosis.

HHS should recognize, however, that these conversational AI systems are moving steadily toward more sophisticated medical reasoning. Microsoft AI has publicly outlined the path toward medical superintelligence, or AI systems capable of far more reliable and transparent clinical reasoning than today's systems. As consumer tools evolve along this trajectory, some functionalities will inevitably approach higher-risk, diagnosis-adjacent intended purposes, thereby requiring careful regulatory consideration that will warrant the appropriate guardrails and oversight.

Preparing for that future now is essential. HHS can lay the groundwork by defining forward-looking regulatory pathways that create an "on-ramp" for currently low-risk consumer AI systems that will eventually bring diagnostic support and other higher-risk capabilities once the technology reaches clinically actionable performance. With appropriate safeguards, longitudinal AI companions can make millions of health queries safer and more accurate, improve comprehension and confidence that strengthens self-management, deepen the connection between patients and clinicians, and support better outcomes across all communities.

As families increasingly turn to AI as their first source of health information, strong federal leadership in shaping safe, scalable pathways is essential to ensuring consumer AI tools can deliver national-level impact while also maintaining consumer safety and trust.

Proactively thinking about how technology will evolve will allow HHS to take a forward-leaning approach that prepares the ecosystem for the next era of AI-enabled healthcare, from today's wellness tools to tomorrow's medical superintelligence.

Conclusion

Microsoft supports HHS's leadership in developing a coherent, forward-looking regulatory, reimbursement and research framework for AI in clinical care. We are committed to partnering with HHS to modernize reimbursement, clarify regulatory boundaries, strengthen interoperability and invest in trusted evaluation infrastructure. With aligned federal action, the United States can deploy consumer and clinical AI that improves outcomes, enhances access, reduces administrative burden and helps empower every patient to take charge of their health and every provider to supply the best care available.